



CORDIPIN

tablets

Nifedipine

Composition

1 tablet contains 10 mg of nifedipine.

Other ingredients: lactose, etc.

Action

Nifedipine inhibits the influx of calcium into myocardial cells and the smooth muscle cells of arteries. It decreases the consumption of oxygen directly by reducing heart work. The oxygen demand is also indirectly decreased as the heart is relieved.

Nifedipine is immediately released from a 10 mg tablet. It is absorbed within 5 to 10 minutes and achieves the maximum concentrations in 1/2 to 1 hour.

Indications

Nifedipine is indicated for the management of hypertension, vasospastic angina pectoris and of chronic stable angina pectoris (effort-associated angina).

Contraindications

Patients hypersensitive to nifedipine or any other ingredient of the drug, and nursing mothers should not take the drug. Patients with cardiogenic shock, advanced aortic stenosis or porphyria should also not be treated with this drug.

Precautions

Especially at the beginning of treatment and if certain drugs intended for treatment of high blood pressure (beta blockers) are taken concomitantly, blood pressure may fall substantially, which some patients tolerate poorly. In patients taking beta blockers, heart failure may occur after the introduction of nifedipine therapy. The doctor will make more frequent appointments for control examinations, which you should pass regularly even if you feel well.

If you suffer from severe heart diseases (hypertrophic cardiomyopathy, unstable angina pectoris), diabetes, severe hepatic dysfunction or severe pulmonary hypertension and you are an elderly person, you will be closely monitored by your doctor during the introduction of nifedipine therapy.

In patients with severe coronary disease, more frequent attacks of angina pectoris may occur at the beginning of treatment.

Patients with unstable angina pectoris or those having recovered from myocardial infarction should not take immediate-release nifedipine. In this case, the doctor should be consulted.

Mild to moderate peripheral edema, typically associated with arterial vasodilation and not due to left ventricular dysfunction, occurs in about one in ten patients treated with nifedipine. This edema occurs primarily in the lower extremities and usually responds to diuretic therapy.

Nifedipine can alter certain laboratory values and tests (mainly liver ones). These changes are not necessarily associated with clinical signs (although cases of cholestasis and jaundice have been reported).

Due to a lack of experience in children, nifedipine should not be used in this age group.

Pregnancy and lactation

Nifedipine should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Nursing mothers should not use the drug.

**Effects on the ability to drive and use machines**

Especially at the beginning of treatment or at concomitant intake of alcohol, nifedipine can, in susceptible patients, transitory decrease the ability to perform dangerous activities, including driving and operating machines.

Interactions with other drugs

Patients should inform their doctor if they take any other drug.

If drugs lowering high blood pressure, beta blockers, drugs increasing urine excretion and certain drugs for treating heart diseases (nitroglycerin and prolonged-release isosorbide) are taken concomitantly, blood pressure is additionally reduced. Fentanyl exerts similar action.

Nifedipine can seriously potentiate the toxicity of magnesium sulphate and induce neuromuscular blockade (jerky movements, swallowing difficulty, paradoxical respiration and neck muscle weakness). Therefore this combination can be hazardous and life-threatening and should be avoided.

Nifedipine may increase serum concentrations of digoxin, phenytoin (probably also carbamazepine) and tacrolimus, and potentiate their action.

Concurrent use of nifedipine and quinidine may cause decreased quinidine concentrations and increased nifedipine concentrations.

Concurrent administration of nifedipine and theophylline can alter the metabolism of theophylline, therefore theophylline serum concentrations should be closely monitored when nifedipine is added, discontinued, or when dosage adjustments are introduced. Dosage adjustments of theophylline may be necessary.

Concomitant administration of nifedipine and rifampicin may cause lowered serum nifedipine concentrations and attenuation of cardiovascular stability (hypertension, exacerbation of angina).

Concurrent administration of nifedipine and cimetidine, triazole antifungals (itraconazole, fluconazole), imidazole antifungals (ketoconazole), cyclosporine, ritonavir or saquinavir may elevate serum nifedipine concentrations and potentiate its action.

During treatment with nifedipine it is not recommended to drink great amounts of grapefruit juice because it can elevate serum levels of nifedipine, thus increasing its effect.

Nifedipine may interfere with the interpretation of pulmonary function tests after a methacholine bronchial challenge. If possible, nifedipine should be discontinued prior to a methacholine test.

Dosage and administration

Dosage is always individualized and directed by the doctor.

The starting dose is one tablet (swallowed whole) 3 times/day. The usual effective dosage range is 10-20 mg three times a day. Doses above 120 mg daily are rarely necessary. More than 180 mg per day is not recommended.

Overdosage

Overdosage could result in excessive peripheral vasodilation with subsequent marked hypotension. Hypotension is usually manifested as nausea, accelerated heartbeat, dizziness, lightheadedness or fainting. In such a case, you should lie down, elevate the legs with a pillow and call the doctor.

If large quantities of tablets are ingested, the signs of intoxication are lowered blood pressure, shock, slow heartbeat or accelerated heartbeat, heart failure and convulsions. After the ingestion of a large number of tablets, the doctor should be consulted as soon as possible so that he can determine the appropriate treatment (gastric lavage, adsorption to activated charcoal and symptomatic treatment).

**Undesirable effects**

Side effects are transient and mild and usually do not necessitate discontinuation of treatment. The most common side effects are dizziness, lightheadedness, headache, fatigue, weakness, facial flushing, heat sensation, peripheral edema (around the ankles, feet, shanks or periorbital), tachycardia, nausea and heartburn. Rarely, transient hypotension may occur.

Less frequently, palpitation, nasal and chest congestion, shortness of breath, diarrhea, constipation, flatulence, joint stiffness, muscle cramps, shakiness, nervousness, jitteriness, sleep disturbances, blurred vision, difficulties in balance, dermatitis, pruritus, urticaria, fever, sweating, chills, sexual difficulties may occur.

Syncope, thrombocytopenia, anemia, leukopenia, purpura, allergic hepatitis, gingival hyperplasia, depression, paranoid syndrome, transient blindness at the peak of plasma level, erythromelalgia and arthritis with positive ANA may also occur.

Nifedipine can alter certain laboratory values and tests (mainly liver ones). These changes are not necessarily associated with clinical signs (although cases of cholestasis and jaundice have been reported).

The patient should inform his doctor about any undesirable effects, even about those which are not mentioned in the insert leaflet.

Storage and expiry date

The drug should not be used after the expiry date given on the packaging.

Do not store over 30°C and protect from light and moisture. Keep out of the reach of children.

Prescribing information

On physician's prescription, only.

Supply

50 tablets of 10 mg

Manufactured by

KRKA, d.d., Novo mesto, Slovenia

Date of revision

6.2016